

CST302: PURSUIT OF WELLBEING
FINAL PROJECT

SECTION 1

SUBMITTED TO: SANJIDA SULTANA

FARDEEN RAHMAN (21316001)

MASHSHARAT WASI TAHA(21216001)

ABEDA ZAHID CHANDRICA (22341061)

This paper explores the **social** and **mental** well-being of three critical yet distinct groups within Bangladesh: **school teachers, nurses, and housemaids**. Drawing upon interviews conducted with members of each community, this text investigates their perceptions of well-being, identifying both shared and divergent experiences. By reviewing existing national policies within the two domains, we seek to understand how these policies align—or fail to align—with the lived realities of these communities. The resultant analysis highlights critical gaps in current frameworks and proposes 5 distinct policies to address legislative shortfalls while hypothesizing 10 potential challenges that will be encountered during implementation.

Policy Review

To begin, we shall review several national policies & edicts pertaining to the wellbeing of the chosen communities; dividing these ordinances into our selected domains of interest. As outlined in the NEP-2010¹, both Iqbal & Nasrullah (2018) and Rouf (2021) note the existence of government proposed student-teacher ratios which, in practice, determine the size of classrooms - underscoring an important factor of the educator's mental health. Additionally, there are legislative provisions in NEP-2010 which aim to promote the status & dignity of teachers in larger society, demonstrating a commitment to social wellbeing (Ministry of Education, 2010). Similarly, in the case of nurses, a 2011 policy raised nursing from a class III to a class II profession in the national employee hierarchy to ensure suitable social status (Joarder et al., 2021). Additionally, DNS (2016) implements guidelines such as family visiting hours, orientations and provides directives on workplace culture, harassment, gender biases etc. to ensure both sociability and "improved morale". Lastly, DWPWP-2015² threatens legal action against employers subjecting housemaids to mental oppression along with advocating in favour of "meetings, seminars, and workshops"; stipulations which nurture mental and social well-being of the maids respectively (BLIS, 2021).

Well-being Perception

Subsequently, an analysis into the nature of well-being perception as exhibited by the chosen communities must be undertaken. Invoking the 5 dimensions of social well-being, our interview findings indicate that in terms of social acceptance, all 3 communities score positively but the housemaid community is especially welcoming; with trustworthiness and helpfulness extending to the realm of financial assistance. Though one explanation for such

¹ National Education Policy 2010

² Domestic Workers Protection and Welfare Policy 2015

camaraderie could be that the strenuous economic conditions faced by the housemaids leads to more cooperation, we could also refer to the interview directly where the housemaid asserts that due to leaving her village for working in the city she sees other workers as her “mothers and sisters” - a familial bond that is absent amongst teachers and nurses. This idea extends to social integration too, where the schoolteacher's interactions are strictly professional, the nurse is comfortable sharing personal issues, and the housemaid attests strong mutual support. Clearly, the “found family” trope is again emergent in shaping community relations; bolstered by the increasingly shared hardships faced within communities as one goes down the socioeconomic ladder from teacher to housemaid. In the case of social contribution, all the communities perceive their work to be integral to the fabric of society though each provides a different perspective on it: the school teacher believes she contributes by nurturing future generations, the nurse claims contribution in terms of public health while the housemaid believes any job (including hers) is a sufficient contribution. One can explain these perspectives by studying each community member’s opinion regarding news-watching: the school teacher watches the news extensively and hence has a broader, posterity based perspective whereas the housemaid who only briefly watched news during the July Movement provides a much narrower point of articulation. In the middle ground, the nurse who has stopped regularly watching the news since July but is a generally informed person, frames her outlook through the present day medical scenario of Bangladesh. On the other hand, if one were to try to explain each community’s news watching habits itself (social coherence) along with the variation in their opinion of the government (social actualization), we would refer to the education level & financial condition of each community where the relatively privileged school teacher has a strong outlook on the government, the less privileged nurse provides a diplomatic and narrower opinion while the underprivileged housemaid is completely apathetic. Examining [dimensions](#) of mental wellbeing, we observe that the nurse exhibits higher resilience than the schoolteacher and housemaid based on the responses to the suicide ideation question. While the latter two have contemplated non-existence, the former emphatically rejected this notion, most likely because the nature of job demands a more critical understanding of mortality. Both the nurse and schoolteacher show similarly modest levels of emotional stability but the housemaid is much worse off, showing signs of mental dejection. This is best explained by the severe physical exhaustion and poor financial conditions faced by her community in particular. Lastly, members of all communities display an abstention to drugs, openness to counselling and positive personal

relationships which indicate mental clarity, perhaps owing to the nature of their employment which requires a certain level of focus, responsibility and social skills.

Policy Gaps

From the policy review portion of this essay, it is blatantly obvious that the most basic laws and policies are very much absent for all of the communities. According to Saumik (2024), the BLA-2006³ does not even consider domestic workers as labourers thus they are deprived of basic rights such as minimum wage & fixed working hours. Additionally, BILS (2023) asserts that domestic workers do not receive formal agreements like appointment letters and almost all of them never get any social security benefits like accident compensation & medical support. The lack of such provisions translate to real life problems like struggling to pay rent, as confirmed in the interview with the housemaid. Along with domestic workers, teachers are also not included in the Bangladesh Labour Act 2006 , thus being deprived of the same basic rights most professions have (Khan et al., 2024). Only teachers working in government schools - which is a small percentage compared to private schools - have better salaries and retirement plans (Khan et al.2024). Even in NEP-2010, only a mere two pages have been dedicated to rights and statuses of teachers, outlining only the working hours for teachers but lacking other equally important policies like medical support, training & leaves. Existing teachers unions are plagued by inner politics to the point that the government is not aware of their issues; a stark example being teachers forced by the government to take online classes during the pandemic - a task they were unprepared for technologically (Khan et al.2024). It can be deduced that such a dire state of teacher's rights was birthed by government negligence coupled with cultural apathy. Moreover, despite their supportive efforts, the interview with the schoolteacher highlights inadequate government subsidised training, as she relies on colleagues for basic classroom control tips. The nurse community, compared to teachers, have to do more work for less pay due to the lopsided ratio of doctors to nurses in Bangladesh which stands at 1:0.75 whereas global healthcare standards recommend a ratio of 1:3 (Rashed et al., 2024). Such shortage is largely due to the cultural perspectives about nurses and lack of proper education. With such an overload of work, most of these nurses experience burnout and job dissatisfaction as it is coupled with the stagnation of career progression as evidenced by a lack of provisions that help nurses advance in their career and allow them to specialize in a fixed field (Rashed et al., 2024). Additionally, the

³ Bangladesh Labour Act 2006

nurse interview highlights the need for policies ensuring security and transportation for nurses working late-night shifts. Lastly, while there exist NGO initiatives to tackle mental health issues, there are no such officially implemented government policies for any of the 3 communities.

Bridging the Gap

Based on the aforementioned legislative shortfalls, we shall now present and discuss the implementation of 5 new policies designed to enhance the mental & social wellbeing of the chosen communities. In particular, all 3 communities are distressed by their respective professions and thus are in dire need of therapeutic mental health counselling. To execute such an initiative on a significant scale (at least 2 counsellors per city ward) would need both government investment and the expertise of NGOs. Instead of a general education on mental health, counsellors should hold specialised knowledge of the 3 communities being discussed - reviewed every 6 months through NGO organised and state funded training programs. After the first year of the initiative, school-teachers & nurses should pay small fees to receive counselling while the housemaids continue to avail it for free; a setup designed to manage cost and reflect the socioeconomic realities of each profession. In the internet age, the sessions could be arranged online subject to agreement between the counsellor and patient. Next, we propose the establishment of paid mental health holidays which would not only shift some of the responsibility to private employers but also raise awareness and reduce cultural stigma. To elaborate, solely relying on state funding and NGO work is unlikely to address core issues of workplace treatment and culture, hence mandating the employers by law to allow 7 paid holidays annually for the specific purpose of mental health would generate a larger societal discussion about the psychological abuse of these communities. This mandate could also be used as a platform to inform the employees about the newly implemented counselling services of the first policy. In terms of social well-being, it is paramount to incorporate all of these communities into the BLA-2006 and set a strict minimum wage policy, enhancing their social integration with the rest of society. Nationwide surveys should be conducted to determine sector-specific minimum wages, factoring in cost of living and compliance should be enforced through inspections and penalties for violators, facilitated by monitoring cells within the MoL⁴ - especially for the informally employed housemaids. Furthermore, another policy should be enacted to impose mandatory

⁴ Ministry of Labour

unionization within each community as this will provide a platform for them to learn about their rights and foster interpersonal connections - bolstering both social actualization and acceptance. Community-wise charters outlining the rights and responsibilities of unions would have to be developed by the MoL along with a state-run ad campaign raising awareness regarding the benefits of unionization. Regular audits should be undertaken by the MoL to ensure unions function transparently and effectively. Finally, we propose the construction of voting booths near schools, hospitals and slums so all of these communities have easier access to vote - facilitating their social contribution. Through this, communities will have an opportunity to usher their own betterment via voting for appropriate candidates. The government could direct employers to organise workshops and discussion forums during voting week to inform their voting - inadvertently increasing social coherence.

Challenges

Considering all our proposed ideas and policies, targeted to boost the mental and social wellbeing of the three chosen communities, we would also like to mention some of the more awaited challenges that the system will face and needs to overcome in the process of implementing them. Taking the first policy in account - availability of mental health counselling for these communities - one of the major hurdles would be to overcome the stigma surrounding mental health awareness. Yes, all three interviewees were on board with the plan, but at that spot, they were lone representatives of their community and were well explained of the issue if they were unaware. But the same cannot be expected when handling mass amounts of these community members. Especially, when coming across older workers or domestic workers in more rural areas - they will be reluctant to participate in any such counselling programs and, additionally, will discourage others of the same community to take part in the name of social norms. The other, more obvious, issue would be the lack of adequate funding and availability of enough trained professionals to be able to cover the underserved areas. Recruiting and retaining more specialized counselors, especially in rural regions, would require high financial assistance. Coming to the holiday mandate policy - ensuring compliance among private employers, particularly those employing domestic workers, would be critical as most of them are unaware of formal labor rights. Another issue might arise when employers resist this policy as it would reduce productivity and instead increase their cost. For the increment of minimum wage policy, although the idea of surveying before is the best possible approach, it will be challenging to address the variation

in the cost-of-living at different places. Additionally, due to the informal employment status of domestic workers, maintaining and inspecting this framework and penalizing violations will require massive government commitment. Similarly, the unionization policy might face resistance due to lack of trust, bonding and awareness amongst all the workers. Instead, to increase outreach and carry out campaigns, they might need to end up spending bulks. Lastly, making more accessible voting infrastructure can have issues like - as most of these people come to the city in order to find a job, their votes might be in their home town or villages not where they work. Besides, constructing booths and spreading awareness will require massive investment of time and funding.

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